

Psychology Flashcards

Ethics & Clinical Psychology

Total Cards: 86

ETHICS

1	STANDARDS FOR ADMISSION	The criteria that psychology licensing boards use to protect the public by determining who may practice psychology.
2	KNOWLEDGE	What the EPPP assesses to evaluate qualifications of applicants for licensure - foundational knowledge for competent practice of psychology.
3	INSANITY	A legal concept that serves as the basis for a criminal defense in many jurisdictions.
4	COMPETENCE TO STAND TRIAL	A defendant's ability to consult with his/her lawyer with a 'reasonable degree of rational understanding.'
5	FACT witness	A witness who testifies to what he/she has seen, heard, or observed regarding an event or occurrence; usually not allowed to offer an opinion.
6	EXPERT witness	A witness qualified by the court based on expertise to offer opinions and respond to hypothetical situations.
7	DUTY OF CARE	First condition for malpractice: a professional relationship that establishes this obligation.
8	STANDARD OF CARE	Second condition for malpractice: a demonstrable benchmark that the psychologist breached.
9	PROXIMATE CAUSE	Fourth condition for malpractice: the psychologist's breach of duty was this of the person's harm or injury.
10	LEGALLY VALID	The first step when responding to a subpoena is to determine if it is this.
11	PROTECTIVE ORDER	A motion that can be filed if a client does not consent to providing subpoenaed information.
12	PSYCHOTHERAPIST-CLIENT PRIVILEGE	What may be asserted in court or at a deposition to protect client information.
13	OLDER	Studies find that male therapists who engage in sexual relationships with clients are typically this compared to the female clients.

14	NON-SEXUAL BOUNDARY crossings	Therapists reporting sexual boundary violations were also more likely to report being involved in these.
15	PRIVACY RULE	HIPAA provision that provides minimum standards for access and use of patient's health information.
16	STRINGENT	State law takes precedence over HIPAA when it is more this.
17	COST-EFFECTIVENESS analysis	Useful when interventions have similar goals but outcomes cannot be assigned a monetary value; expressed in measurable terms like symptom severity scores.
18	COST-UTILITY analysis	Compares interventions by comparing costs with outcomes in terms of quality-adjusted life-years (QALYs).
19	MASCULINE PRONOUN	The Publication Manual recommends avoiding the generic use of this by using plural forms or omitting when confusion is unlikely.
20	PHYSICAL OR MENTAL DISABILITY	The Publication Manual recommends putting people first when describing people with this.

CLINICAL PSYCHOLOGY #1

21	ID	One of Freud's three personality structures; consists of a person's instincts and serves as the source of all psychic energy.
22	4 OR 5	The age at which the superego develops according to Freud.
23	PSYCHOSEXUAL	Freud's developmental theory proposes five of these stages of development.
24	LIBIDO (SEXUAL ENERGY)	During each psychosexual stage, the id's energy is centered in a different part of the body.
25	EGO	Anxiety alerts this to internal or external danger related to conflict between the id and superego or reality.
26	REACTION FORMATION	A defense mechanism that involves avoiding an anxiety-arousing instinct by expressing its opposite.
27	INTERPRETATION	One of the four components of psychoanalysis, along with confrontation, clarification, and working through.
28	COUNTERTRANSFERENC E	Recent modifications consider the therapist's this to be an important source of information about a patient when recognized and managed appropriately.
29	SOCIAL	Adler emphasized this factor's role in personality development.
30	TELEOLOGICAL	Adler's approach that views behavior as motivated largely by future goals.
31	STYLE OF LIFE	According to Adler, this reflects ways a person chooses to compensate for feelings of inferiority and achieve superiority.
32	PERSONAL unconscious	According to Jung, contains material passed down from one generation to the next.
33	ARCHETYPES	The self, the persona, and the shadow are examples of these that have particular importance for personality development.
34	OBJECT-SEEKING	Object relations theorists consider this to be the basic inborn drive.

35	SEPARATION-INDIVIDUATION	Mahler's process that begins at about four or five months of age; problems during this phase are attributed to many forms of psychopathology.
36	SPLITTING	According to Kernberg, Borderline Personality Disorder is due to inadequate resolution of this of objects into 'good' and 'bad' components.

CLINICAL PSYCHOLOGY #2

37	SELF-ACTUALIZE	Rogers believed all people have an inherent tendency to do this.
38	SELF	Incongruence between this and experience interferes with self-actualization.
39	POSITIVE REGARD	Unconditional acceptance of a client without evaluation; one of three facilitative conditions.
40	ACCURATE EMPATHY	The ability to understand the world as the client does; one of three facilitative conditions.
41	GENUINENESS	Provided when a therapist honestly communicates his/her feelings when appropriate; one of three facilitative conditions.
42	INTEGRATED	Gestalt therapy assumes each person is capable of living fully as this kind of whole.
43	SELF-IMAGE	Neurotic behavior occurs when the individual abandons the self for this.
44	INTROJECTION	A boundary disturbance that occurs when a person psychologically 'swallows' whole concepts from the environment without fully understanding them.
45	THOUGHTS	Gestaltian awareness involves a full understanding of one's this in the here-and-now.
46	ULTIMATE CONCERNS	For existential therapists, maladaptive behavior is due to inability to cope authentically with these (death, freedom, existential isolation, and meaninglessness).
47	NEUROTIC anxiety	Often results from an attempt to avoid existential anxiety.
48	LOVE AND BELONGING	One of Glasser's five basic needs, along with survival, power, freedom, and fun.
49	SUCCESS identity	When people fulfill their needs in a responsible way, they have adopted this.
50	FAILURE identity	When people gratify their needs in an irresponsible way, they have assumed this.

51	BIPOLAR	Personal constructs are these dimensions of meaning that begin to develop in infancy.
52	FIXED-ROLE THERAPY	Kelly's technique to help clients 'try on' and adopt alternative personal constructs.

CLINICAL PSYCHOLOGY #3

53	SYMPTOM REDUCTION	One of the two primary goals of interpersonal therapy, along with improved interpersonal functioning.
54	ROLE TRANSITIONS	One of four problem areas addressed by interpersonal therapy interventions.
55	EXCEPTION questions	Solution-focused therapy questions that help identify when problems don't occur.
56	STAGE OF CHANGE	The transtheoretical model assumes the best interventions match the client's this.
57	PRECONTEMPLATION	The first stage of change in the transtheoretical model.
58	CONTEMPLATION	The stage of change where a person intends to take action within the next six months.
59	SIX months	The minimum time a person has maintained behavior change to be in the maintenance stage.
60	CLIENT-CENTERED THERAPY	Motivational interviewing combines the transtheoretical model with elements of this.

CLINICAL PSYCHOLOGY #4

61	OPEN system	How family therapy models view the family - receiving input from and discharging output to the environment.
62	NEGATIVE	This type of feedback loop provides information that helps the family system maintain the status quo.
63	LOCKEAN	Western scientific tradition that emphasizes linear cause-effect relationships and individualism.
64	RECIPROCAL	Family therapies reflect this view of causality and a relational, contextual, relativistic perspective.
65	ACKERMAN	Child psychiatrist known as the 'grandfather of family therapy.'
66	DOUBLE-BIND	Type of communication Gregory Bateson studied in relation to schizophrenia development.
67	EQUAL	In symmetrical communication, participants are this.
68	COMPLEMENTARY	In this type of communication, participants are unequal and differences are maximized.
69	FUSE	When family members have low levels of differentiation, they easily do this with other members.
70	TENSION	Emotional triangles develop when a two-person system recruits another to increase stability and reduce this.
71	GENOGRAM	Bowenian assessment tool that depicts relationships between family members.
72	EMESHED	When boundaries are overly diffuse, family members are this.
73	STABLE COALITION	Occurs when a parent and child consistently 'gang up' against the other parent.
74	JOINING	First step in structural family therapy; entails 'blending' with the family by adopting their style.
75	COMMUNICATION	Strategic family therapy emphasizes this role in maladaptive behavior.

76	RESISTANCE	Paradoxical interventions are designed to use a client's this in a constructive way.
77	ORDEAL	An unpleasant task a client must perform when engaging in symptomatic behavior.
78	REFRAMING	Relabeling a symptom to give it a more positive meaning.
79	PERCEPTIONS	Circular questions help family members recognize differences in their this.
80	ANTECEDENT AND CONSEQUENCES	Behavioral family therapies alter these that maintain problematic behaviors.
81	COMMUNICATION AND PROBLEM-SOLVING	Primary focus of behavioral family therapies regarding family members' skills.
82	INTRAPSYCHIC AND INTERPERSONAL	Object relations family therapists view maladaptive behavior as the result of both these factors.
83	INTROJECTS	Goal of object relations family therapy is to resolve each member's attachment to family this.

CLINICAL PSYCHOLOGY #5

84	COHESIVENESS	In Yalom's third stage of group therapy, this develops, leading to increased self-disclosure and improved attendance.
85	PARTICIPANT/MODEL	One of two roles the therapist adopts to establish group norms (along with technical expert).
86	SELF UNDERSTANDING	One of the curative factors members rate as most important, along with interpersonal input, catharsis, and cohesiveness.